TRAUMA-06 | For therapists | Two separate age-based pathways

Avoidance and applying learning in everyday life

1 | Purpose, fit and boundaries

Purpose of this unit

Record with the therapist situations where difficulty limits activity and support agreed application in life. Distinguishing past reminders, practical difficulty and current danger is part of professional assessment.

Choice and adaptation

Choose this sheet following professional assessment and with consent. The person may skip questions, answer aloud, or choose another tool. This is proposed original material for review; it provides no score, diagnosis, or promise of an outcome.

Boundaries specific to this unit

The form neither establishes environmental safety nor gives exposure instructions. Do not select dosage, duration or hierarchy here. Avoiding current danger is not treatment failure; do not dismiss medical limits or protection needs.

Reusing existing material

ANX-04 does not replace trauma-reminder work. EXEC may aid practical planning. Unit 6 records decisions and learning within the chosen model, not a new exposure pathway.

A fictional example for discussion

Fictional example: Oren wants to return to a group activity. Discussion reveals a reminder-related difficulty and transport barriers. The therapist assesses both before agreeing any follow-up.

> Draft for administrator review. Clinical review and final rights approval are still pending.

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Working together with the worksheet

2 | Age-based pathways

CH / TE | Support within a TF-CBT pathway

Work with real-life reminders, when suitable within TF-CBT, follows the manual and professional assessment. CH/TE sheets support exploration and recording only; they set no exposure sequence or dosage.

AD / OA | Support within a CPT pathway

AD/OA sheets explore meaning and life participation within CPT. Separate belief exploration from claims of environmental safety and from medical or practical needs.

Parallel work with a suitable support person

Agree on a limited, clear supporter role chosen in treatment. They do not pressure, surprise or change the task. Harm or environmental changes require professional assessment and protection.

Using W and P sheets

Choose the age version and relevant questions; completing every sheet is not a condition for progress. W accompanies professional conversation. P records only pre-agreed follow-up. Unit number does not mean session number.

Closing

Summarize what became clearer and what remains open. Use the existing SES-S01 and SES-S02 for the session framework. There is no need to repeat an assessment or plan already recorded.

Possible difficulties

If an action was not done, explore barriers without labeling disobedience. Fear, current danger, fatigue and practical barriers differ; changed conditions require reassessment.

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Adapting for four age groups

3 | Age is a starting point, not an assumption about ability

CH | Children, approximately ages 6 to 11

In CH, the therapist and suitable supporter assess context; the child is not responsible for proving safety. The sheet allows voice and choice within a professional plan.

Offer one question at a time with little writing. Adult support must not replace the child's own voice.

TE | Adolescents, approximately ages 12 to 17

TE allows for independence and peer pressure, distinguishing personal choice, fear and real danger. Do not ask friends to manage a treatment experiment instead of the therapist.

Agree in advance what will be shared and with whom, and explain the privacy limits in this setting. Do not promise absolute confidentiality.

AD | Adults

AD explores how a belief examined in CPT affects life participation. This sheet does not turn the pathway into exposure treatment or independently assign behavioral tasks.

OA | Older adults

OA distinguishes limitations from reminders or meanings versus access, health and environmental barriers. Do not assume every avoidance reinforces difficulty; choose adaptations valuable to the person.

Offer reading support, time, and conversation according to preference. Choose help from another person together with the participant, respecting their privacy.

> Each age version needs its own review. Approval of one version does not automatically approve the others.

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Practice, review and next steps

4 | Shared choice and learning from the attempt

Before offering practice sheet P

P records support and learning agreed in treatment. It sets no exposure, trauma narrative, dosage or component sequence. Without a plan, bring questions to the session. CH/TE sheets do not replace separate supporter work; AD/OA sheets are not official CPT materials.

At the next session

Compare agreements, events and learning. One uneventful attempt does not establish safety; do not automatically enlarge tasks. Update the plan together.

Professional and technical records

Each version has a separate review record covering age suitability, originality, rights, and fictional filling. Reviewer and date fields remain blank until assigned. File checks or a code merge are not clinical approval. Clinical information stays only in the authorized setting, outside the code repository.

Background references

The following links were used for background only. Questions and adaptations were written afresh for the expansion plan; no questionnaire, scale, or protocol was copied. Adult sources do not validate the child versions. This material makes no claim about session count or treatment effectiveness.

Treatment-manual mapping status

Proposed: TF-CBT for children/teens; CPT for adults/older adults. Full manuals remain unmapped. Age, rights, supporter materials and separate-guide needs await review. Original support drafts, not an approved protocol.

www.nctsn.org/interventions/trauma-focused-cognitive-behavioral-therapy
tfcbt.org/about/
www.ptsd.va.gov/professional/treat/txessentials/cpt_for_ptsd_pro.asp

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